WHITE ARMY COMPREHENSIVE CLINICAL CLASS

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Patient particulars

- Name: Mr. ABC
- Age: 32 years
- Sex: Male
- Education: B.E.
- Occupation: engineer
- Socio-economic status: class 2
  (Modified BG Prasad Classification)
- Address: Naragund
- D.O.A: 12/02/2020
- D.O.E: 13/02/2020

Chief Complaints

- Pain in upper abdomen since 1 week
- Nausea and vomiting since 1 week
- Jaundice since 1 week

History of presenting illness

- Patient was apparently alright 2 years ago. Then he developed recurrent pain in the upper part of abdomen radiating to back. The current episode is from 1 week and was acute in onset. It is continuous dull aching type.
  - Aggravates on taking fatty food
  - Relieved by leaning forward
  - The pain has no periodicity

- On some occasions the pain became sudden, severe and agonizing in nature and didn’t relieve by taking analgesics. He was admitted to the hospital for the same, 3 times in the past 2 years and was treated conservatively.
- Patient also complains of frequent nausea and vomiting, copious in amount, bilious in nature, not related to food intake.

- History of jaundice
- History of on and off fever present
- History of weight loss (9 kgs in 2 years)
- History of loss of appetite
- History of constipation with fatty and offensive stools
- He is diabetic since 6 months

- No history of stomach cramps
- No history of hematemesis or malaena
- No history of trauma to the abdomen
- No history of signs of liver failure
- No history of heart burn, chest pain
- No history of respiratory distress

Past history

- Known case of diabetes mellitus (6 months) on medication
- No history of any chronic illnesses like hypertension, asthma, tuberculosis, epilepsy
- No history of any surgery of the abdomen

Treatment history

- Admitted to hospital 3 times due to similar complaints and was managed conservatively.

Allergy history

- No known allergies till date

Family history

No history of similar complaints in the family

No history of asthma, hypertension, DM, TB, epilepsy

Personal history

- Diet: vegetarian
- Sleep : disturbed
- Appetite: decreased
- Bowel movements: constipation with fatty and offensive stools
- Bladder habits: normal and regular
- No history of any substance abuse

Summary

- A 32 year young man, who is a known case of diabetes mellitus for 6 months, presented with epigastric pain radiating to back, with severe nausea and vomiting, and jaundice since 1 week.
- He has history of recurrent symptoms for 2 years along with weight loss and fatty and offensive stools
- All these features are consistent with pancreatic involvement.

Physical Examination

- Here is a young gentleman moderately built and poorly nourished, conscious cooperative and well oriented to time, place and person. Patient looks dehydrated.
- Gait: normal
- Colour of skin: normal
- Attitude: leaning forward
- Icterus – present
- No Pallor
- No Cyanosis
- No Clubbing
- No Lymphadenopathy
- No Edema

Vitals

- Pulse : 110 beats/min, normal volume, no vessel wall thickening
- Blood pressure: 120/80 mm hg
- Respiration: 13 cycles per minute, abdomino-thoracic type.
- Temperature: 37 degree celsius

Per Abdomen Examination

Inspection:

- Patient is lying flat on the bed with legs extended and adequately exposed and examination carried out in good day light
- Skin – normal
- No engorged veins
- Umbilicus is central and inverted
- Corresponding regions move equally with respiration
- Hernial orifices intact
- No scars
- No swellings on the abdomen
- Genitalia are normal

Palpation:

- Patient was asked to lie down on back comfortably and flex his knees to release the abdominal muscles. Patient was adequately exposed
- No local rise of temperature
- Guarding is present
- Tenderness on deep palpation of left subcostal and epigastric regions

Percussion

- Resonant all over abdomen except liver and spleen dullness. There is no obstruction to liver dullness

Auscultation

- Decreased or absent bowel sounds

Examination of other systems

- **Respiratory system**
  - Bilateral vesicular breath sounds are heard
- **CVS**
  - Normal heart rate and regular rhythm
  - Heart sounds S1 and S2 heard
- **CNS**:
  - No neurological deficits present
- **Skeletal system**
  - No bony tenderness or evidence of fracture [due to accumulation of calcium in pancreas – hypocalcemia]

Summary

- 32 year young man presented with epigastric pain radiating to back with severe nausea, vomiting, weight loss, fatty offensive stools, and diabetes mellitus.
- On examination icterus, tenderness in epigastric and left subcostal region was present.

Provisional diagnosis

- It is a case of Pancreatitis

Differential diagnosis

- Hepatitis